The Financial Opportunity Cost of Continued Navy Service

A General Internist Case

James M. Day

October 19, 2026

BUS 620 - Micro and Macro Economics

Individual Research Paper - a global or local challenge, analyzed

Executive summary

The financial cost of staying in the Navy depends on what is counted. For a modeled general internist at 10 years of service, the cost of staying to retirement falls from about $588,000 on gross compensation to $39,000 after taxes and benefits. This single base case weights pension payments by approximate survival and assumes no promotion, isolating the compensation comparison from promotion timing. Promotion is a separate sensitivity, not an alternative headline. The model does not show what physicians believe or whether education changes retention. A command-randomized Career Development Board (CDB) pilot should test compensation knowledge and retention while higher-level clinical skill-sustainment work continues. Current Navy internist-specific staffing and departure data remain an evidence gap.

Challenge and approach

Across Department of Defense (DoD) medical facilities, assigned military medical personnel fell from 60,479 in FY2015 to 50,590 in FY2023 (U.S. Government Accountability Office [GAO], 2025). This describes staffing pressure, not Navy internist departures. GAO (2020) found military cash pay below civilian medians in 21 of 27 medical and dental specialties. No current public Navy internist-specific retention or fill rate was identified in the sources reviewed; specialty-leader or Bureau of Medicine and Surgery (BUMED) data are needed.

Opportunity cost is what the physician gives up by staying; past medical-school support is a sunk cost. Compensating wage differentials reflect job amenities and burdens (Frank et al., 2019). This case compares a married, age-37 lieutenant commander (O-4) in San Diego leaving at 10 active-service years with staying to 20. Commander is O-5. Marit Health's (2026) $459,057 average is a benchmark, not a matched offer. Appendix A specifies inputs. Cash flows use a 5% real discount rate and pensions 3%; amounts are constant 2026 dollars.

Results and their meaning

One base case is used throughout: no promotion, California wages, and pension payments weighted by approximate survival from retirement at age 47. The ten-year gross civilian pay advantage is $1,349,196. A conditional gross pension worth $761,204 offsets part of it, leaving a $587,992 cost of staying. Appendix B shows the annual pay comparison; Appendix D shows the pension calculation.

Tax-free housing and subsistence allowances favor Navy pay, while civilian wages face income and payroll taxes. Adding employer retirement contributions and subtracting civilian insurance and health premiums reduces the cost of staying to $39,395. This is tax-adjusted compensation, not spendable income: retirement contributions enter at face value, and later withdrawal taxes are omitted. Appendix C gives the tax simplifications. Figure 1 shows break-even near 14.1 service years on the gross basis and 10.6 years after taxes and benefits.

No promotion is a lower-pay scenario chosen to isolate longevity and pension value without assigning an individual selection probability or promotion date. It is not a forecast of a typical career. The FY27 Medical Corps commander board selected 84 of 115 in-zone officers (73.04%; Navy Personnel Command, 2026). That is a one-board cohort rate, not an individual lifetime probability. Promotion raises the value of staying; under the illustrative timing in Appendix E, staying becomes roughly break-even or better. Other promotion and pension-tax sensitivities remain in that appendix.

Counting taxes and benefits shrinks the cost of staying from about $588,000 to about $39,000. Figure 2 uses this same base and shows how additional conditional benefits can narrow or reverse the difference. These scenarios are not a confidence interval or proof that staying is financially superior. Retirement eligibility remains assumed; the survival schedule is an approximation recovered from remaining life expectancies, not a directly observed mortality schedule.

Policy implications and existing services

The modeled gap raises a testable question: do physicians misjudge total compensation, and does a review improve understanding? The department's Beyond Basic Pay campaign urges leaders and families to discuss the full value of military compensation, showing that the department treats this as an education need (Office of Military Compensation and Financial Readiness, 2026). That supports the pilot premise, not a finding about internists. GAO (2005) documented historical misunderstandings, but its focus groups were not representative. A current internist knowledge gap remains a hypothesis to measure at baseline.

Clinical skill sustainment remains essential. The Department of Defense Office of Inspector General (DODIG, 2025, p. 15) reports that a January 2024 BUMED emergency-physician study identified skill-degradation concerns as the leading factor in junior-officer attrition. That finding cannot be generalized directly to internists. DODIG recommends using wartime readiness requirements in assignments and prioritizing military-civilian partnerships. These changes require higher-level decisions and partnership capacity; a command can pilot a CDB review now, measure knowledge at six months, and evaluate retention 24 months after obligation completion. The review can therefore start while that structural work continues; it does not replace it.

The Medical Corps already expects O-3 through O-5 physicians, excluding interns, to complete a Career Development Board (CDB) each tour (Schofer, 2025). Navy financial counseling and special-pay guidance provide existing resources. NHS communication materials offer adaptable examples, but broader retention efforts and retirement-plan experiments do not establish a physician-retention effect (NHS Employers, 2020, 2022, 2025; Goldin et al., 2020; Duflo & Saez, 2003).

A review is proposed as a low-cost addition, with actual costs measured. If knowledge improves without retention, attention should return to pay and working conditions. Appendix E contains the numerical sensitivities. Unmatched offers, deductions, retirement-account taxation, separation risk, and nonfinancial preferences limit individual conclusions.

Recommendation and decision rule

Pilot a standardized compensation review within the Medical Corps CDB program for physicians whose obligations end within 12 months. Require enhanced-group commands to schedule and document an offer before extension or separation commitment. The CDB coordinator arranges access, financial specialists provide counseling, and personnel specialists verify pays and obligations. Keep household finances and civilian offers private, with voluntary disclosure. Appendix F defines the proposed roles.

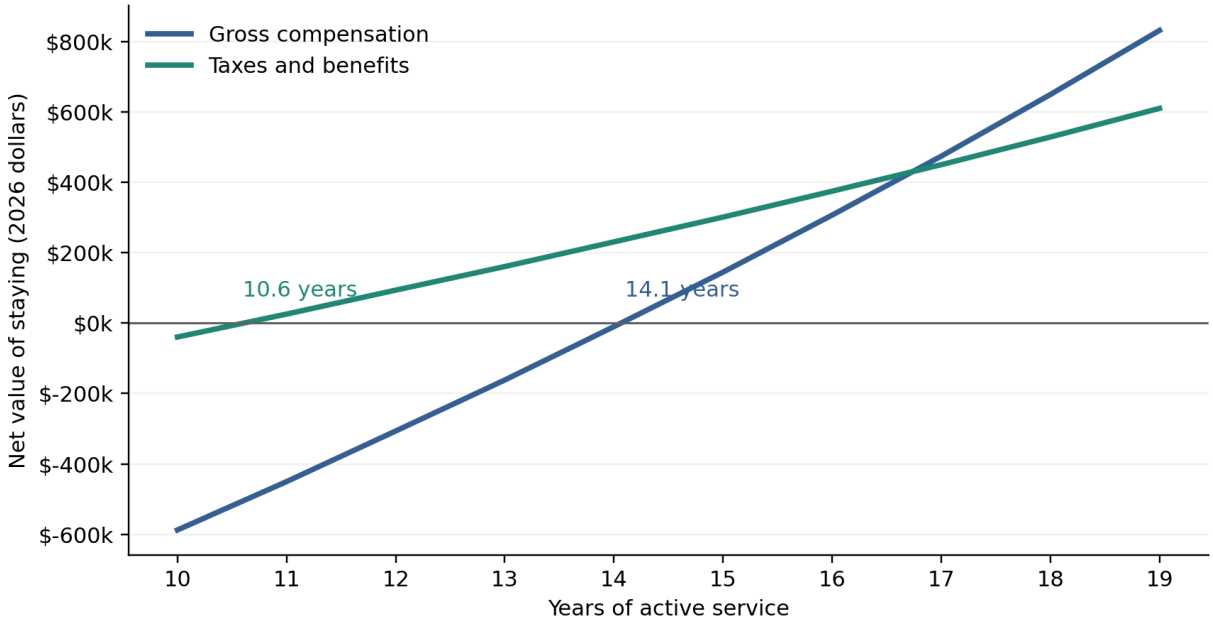
Randomize commands to existing CDB and financial services or those services plus the standardized review. Balance specialty and service stage; analyze by assigned group with command clustering. Preserve usual services and record their use. Measure knowledge at baseline and six months, and active-service status 24 months after the original obligation ends. Compare baseline estimates with verified compensation; intent to leave is not evidence of misunderstanding. Calculate sample requirements prospectively; begin with feasibility and comprehension if recruitment is insufficient.

Before enrollment, approve proposed expansion criteria: a five-percentage-point retention improvement, a 95% confidence interval excluding zero, and cost per additional physician retained below 25% of documented specialty replacement cost. These are policy judgments. Include CDB coordination, counselor time, eligibility checks, model updates, administration, and evaluation. Existing infrastructure does not prove low cost. Continue assignment and clinical-practice work in parallel. If knowledge improves without retention, reconsider pay and working conditions. Report inconclusive effects without claiming success.

Job satisfaction, professional identity, family preferences, deployment stress, and commitment to service may outweigh money. An informed decision to separate is also a valid counseling outcome.

Figure 1. Break-even depends on what is counted

Single base: no promotion and approximate survival-weighted pension.

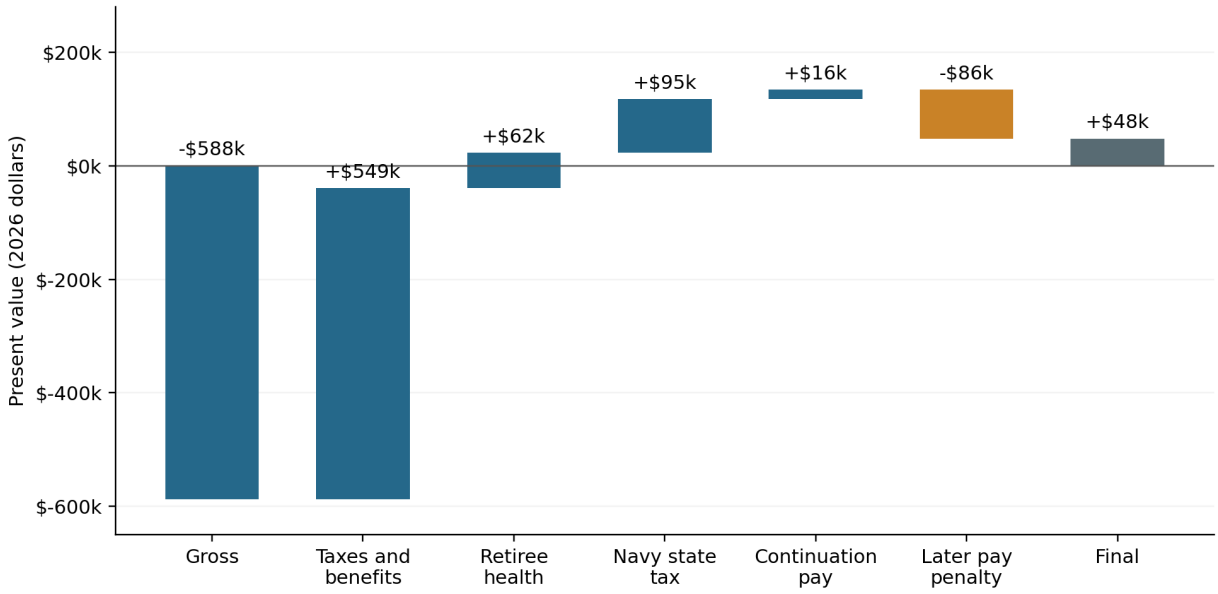


Positive values favor staying. The same O-4 longevity pay and annual pension of $50,274 apply to both paths. Future pension payments are weighted by approximate survival from age 47 and discounted at 3%; compensation gaps use 5%. One-year survival is recovered from the shared remaining-life-expectancy proxy with a curtate approximation. Retirement eligibility is assumed, not probability-weighted. The adjusted line excludes retiree health, continuation pay, and later civilian earnings differences; crossings are interpolated, not retention effects.

Sources: Appendices A-E and appendix_calculations.py; data/us-life-remaining-2024.json. Inputs and calculations accessed October 19, 2026.

Figure 2. Taxes and benefits narrow the difference

Present value at 10 service years; same survival-weighted pension base as Figure 1.



Each step adds to the preceding result. Positive values favor staying. Navy state tax means a qualifying domicile without state income tax during active duty. Later pay penalty means 5% lower civilian pay after military retirement, through age 65. Pension payments are survival-weighted; the health and later earnings scenarios retain fixed age-47-to-65 horizons and do not add mortality adjustments. Values are rounded; exact totals and assumptions appear in Appendix D. Sources: Appendices A-E; calculations accessed October 19, 2026.

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Appendix A Compensation and assumptions

Table A1. O-4 annual cash compensation by completed service years. BAH is $5,082 per month for ZIP 92134 with dependents. BAS is $328.48 per month. Incentive pay, board certification pay, and retention bonus total $99,000 per year.

Completed years	Basic pay	Navy gross cash
10-11	$113,040	$276,966
12-13	$118,660	$282,585
14-15	$122,573	$286,499
16-17	$124,819	$288,745
18-19	$126,119	$290,045

Sources: DFAS (2026a-c), DTMO (n.d.). Pay rates are held in 2026 dollars. The $48,000 annual retention bonus assumes continued eligibility and agreements covering the full comparison horizon; it is not guaranteed. No promotion is assumed.

Civilian employee cash pay is $459,057 each year (Marit Health, 2026). Current age is 37; retirement occurs at 47; work ends at 65. Married filing jointly, one income, no child credits. No VA benefits, survivor coverage, pay growth, transition cost, or first-year pay reduction is included.

Appendix A Benefit assumptions continued

Item	Annual amount or assumption
BRS service contribution	5% of basic pay; full eligibility and employee contributions assumed
Civilian employer match	$16,560 = 4.6% of min($459,057, $360,000)
Civilian worker health premium	$6,850 family premium share
Private disability insurance	$13,771.71 = 3% of civilian pay
Retiree health sensitivity	$6,085 = $6,850 avoided premium less $765 fee
Continuation pay sensitivity	$23,550 in comparison year 2; 2.5 months of basic pay

Sources: DoD (2017), Vanguard (2025), IRS (2025b), KFF (2025), TRICARE (2026), and The White Coat Investor (n.d.). Continuation pay is a conditional scenario assumption; eligibility, future rates, and obligation terms must be confirmed.

Employer contributions are compensation value, not current cash. Their future withdrawal taxes and employee contribution deductions are excluded. Insurance at 3% is an illustrative quote proxy. Health savings ignore deductibles, copays, and differences in access. These assumptions require individual verification.

Appendix B Annual adjusted comparison

Table B1. Dollars rounded for display; calculations use unrounded values. Civilian adjusted compensation is $305,005.33 every year. Navy tax includes federal, California, and employee payroll taxes. Navy value equals gross cash minus taxes plus the BRS service contribution.

Year	Navy gross	Navy tax	Navy value	Civilian gap
1	276,966	55,069	227,549	77,457
2	276,966	55,069	227,549	77,457
3	282,585	56,909	231,609	73,396
4	282,585	56,909	231,609	73,396
5	286,499	58,191	234,436	70,569
6	286,499	58,191	234,436	70,569
7	288,745	58,927	236,059	68,946
8	288,745	58,927	236,059	68,946
9	290,045	59,352	236,998	68,007
10	290,045	59,352	236,998	68,007

The present value of the annual civilian gaps at 5% is $557,014.21. The corresponding gross gaps total $1,349,196.04 in present value. For the first four years, the gross-gap present value is $636,208.84. All payments occur at year end. Tax and civilian benefit components are specified in Appendix C.

Appendix C Tax calculation inputs

Federal income tax uses 2026 married-filing-jointly brackets and a $32,200 standard deduction (IRS, 2025a). California uses the published 2025 Schedule Y and $11,080 standard deduction as a proxy held constant; these are not verified 2026 California brackets (FTB, 2025a, 2025b).

Marginal rate	Federal lower bound	California lower bound
10% federal / 1% CA	$0	$0
12% / 2%	$24,800	$22,158
22% / 4%	$100,800	$52,528
24% / 6%	$211,400	$82,904
32% / 8%	$403,550	$115,084
35% / 9.3%	$512,450	$145,448
37% federal	$768,700	Not used

Apply each marginal rate only to taxable income within its bracket. California income remains below the next $742,958 threshold in all reported cases. State exemption credits, itemization, SALT interactions, and alternative minimum tax are excluded. The model is an illustration, not a tax return.

Appendix C Worked example and payroll taxes

Year 1 Navy taxable wages are $276,965.76 minus $64,925.76 in BAH and BAS, or $212,040 (IRS, 2025c). Civilian wages are $459,057. Taxable income for each tax is wages minus that tax system's standard deduction.

Year 1 component	Navy	Civilian
Federal income tax	$28,989	$89,506
California income tax	$11,567	$34,539
Employee payroll tax	$14,514	$25,945
Total modeled taxes	$55,069	$149,990

Payroll tax = 6.2% of wages up to $184,500, plus 1.45% of all wages, plus 0.9% above $250,000 for this joint household. California civilian wages also incur 1.3% SDI (SSA, 2026; EDD, 2026). Military allowances are excluded from wages.

Civilian value = $459,057 - $149,989.96 + $16,560 - $6,850 - $13,771.71 = $305,005.33. Navy year-1 value = $276,965.76 - $55,068.94 + $5,652 = $227,548.82. Difference = $77,456.51.

Appendix D. Pension and present value

The single base discounts each future pension payment by 3% and weights it by approximate survival from retirement at age 47. Let S(t) be survival to the end of retirement year t. The annuity factor is the sum of S(t)/(1.03)^t across the shared series, or 20.34829551. For n years until retirement, pension PV = annual pension times this factor, divided by (1.03)^n.

High-3 basic pay = ($124,819.20 + $126,118.80 + $126,118.80) / 3 = $125,685.60. The BRS annual pension is 2% times 20 years times High-3 = $50,274.24 (DoD, n.d.-a). Payments begin one year after retirement.

At 10 service years, gross pension PV is $761,204.42. Subtracting the gross compensation gap of $1,349,196.04 gives -$587,991.62. A flat illustrative 32% pension tax gives pension PV of $517,619.01; subtracting the adjusted compensation gap of $557,014.21 gives -$39,395.20.

At each later service year, discard completed compensation years and discount the remaining gaps from that decision date, keeping the same retirement pension. Interpolation gives break-even at 14.06 years gross and 10.60 years adjusted.

The survival proxy recovers p(x) = [e(x)-0.5]/[1+e(x+1)-0.5], capped at 1, from remaining life expectancy e(x); cumulative products give S(t). It approximates mortality, not the probability of reaching pension eligibility. No separate separation or pension-eligibility probability adjustment is made in this base case. A constant pension tax is also a simplification. Appendix E tests other assumptions.

Sources: appendix_calculations.py; data/us-life-remaining-2024.json; Appendix A. Accessed October 19, 2026. Retiree health and later earnings scenarios use fixed horizons, as stated in Table D1.

Appendix D. Cumulative adjustments

Table D1. Net value of staying at 10 service years, constant 2026 dollars.

Comparison	Net value
Gross compensation; untaxed pension	-$587,992
Taxes and current benefits	-$39,395
Add retiree health	$22,878
Add no-income-tax Navy domicile	$117,480
Add assumed continuation pay	$133,832
Add 5% later civilian pay penalty	$48,096

Each row adds the stated change to the preceding row; positive values favor staying. All rows use the same no-promotion, survival-weighted pension base, longevity schedule, High-3, and 32% illustrative pension tax.

Retiree health PV = $6,085 times A(0.03,18) / (1.03)^10 = $62,273.31, where A(r,n) = [1-(1+r)^(-n)]/r. Active-duty Navy state income tax removal saves $94,602.25. Conditional continuation pay contributes $16,351.50 after incremental federal and payroll tax, discounted two years.

The later earnings penalty costs $85,736.06. It compares civilian pay of $459,057 and $436,104.15 after modeled wage taxes, employer match, and 3% insurance, for 18 years after retirement. Fixed health premiums cancel. These scenarios are not additive beyond the displayed sequence.

Only pension payments are survival-weighted. Health and later earnings assumptions retain fixed horizons to age 65; these scenario increments are not a complete mortality-contingent lifetime model. Sources: Appendices A-C and appendix_calculations.py; accessed October 19, 2026.

Appendix E. Sensitivity and model boundaries

Table E1. One base and two comparisons (2026 dollars)

Pension / promotion assumption	Net gross	Net adjusted
Base: O-4, survival weighting	-$587,992	-$39,395
Comparison: O-4, 30 fixed payments	-$615,968	-$58,419
O-5 at 15 years, survival weighting	-$449,810	$61,585

The fixed 30-payment horizon is retained only as this comparison line. Promotion includes higher pay, housing allowance, taxable wages, and High-3. No promotion is a lower-pay scenario, not a typical-career forecast or universal financial floor. With promotion at 15, the annual pension is $55,710; the adjusted result already favors staying at 10 service years.

FY27 commander selection, Medical Corps: 84 selected / 115 in-zone = 73.04%; above-zone 24/53 = 45.28%; below-zone 1/328 = 0.30%. The printed 109/115 = 94.78% divides all selectees by the in-zone population, so it is not the in-zone selection rate. These one-board data do not identify an internist's lifetime promotion probability or timing (Navy Personnel Command, 2026, p. 1).

Table E2. Other one-input changes to the survival-weighted adjusted base

Change	Net adjusted
Base: pension discount 3%	-$39,395
Pension discount 5%	-$233,152
Civilian salary $400,000	$220,049
Civilian salary $350,000	$455,446
Pension tax 41.3%	-$110,187

Lower salary also changes modeled taxes and insurance. Pension tax 41.3% adds an illustrative 9.3-point state burden. The survival proxy recovers a 20.34830 annuity factor at 3% from remaining life expectancies; it is not a validated mortality schedule. Unmatched offers, workload, account taxation, separation, and personal preferences remain limitations.

Sources: Navy Personnel Command, FY27_AO5S_STATS, May 7, 2026, p. 1; appendix_calculations.py; shared calc-engine.js, data/bah-2026.json, and data/us-life-remaining-2024.json. Accessed October 19, 2026.

Appendix F Existing services and the pilot gap

Medical Corps Career Development Boards (CDBs) provide the delivery foundation. A December 2025 announcement reiterates the once-per-tour expectation for O-3 through O-5 physicians, excluding interns (Schofer, 2025). The pilot adds a standardized financial review timed to obligation completion and a retention evaluation. That added requirement is proposed, not existing policy.

Financial counseling. Navy milestone training and individual counseling already cover finances; continuation-pay counseling addresses eligibility and obligations. The pilot adds a standardized stay-or-leave comparison (Commander, Navy Installations Command, n.d.; Financial Readiness Program, n.d.; MyNavy HR, n.d.-a).

Compensation information. The Beyond Basic Pay campaign explains total rewards, and Navy Medicine publishes special-pay guidance. The pilot applies these to household taxes, specialty pay, pension timing, and civilian offer terms (Department of Defense, n.d.-b; Navy Medicine, n.d.). DFAS (2021) directs Navy personnel to the RMC calculator; an annual Navy myPay compensation statement is not assumed.

Transition preparation. TAP initial and pre-separation counseling generally begin at least 365 days before release. Early timing already exists; the pilot focuses on evaluating continued service before commitment, while preserving access to TAP (MyNavy HR, n.d.-b).

Retention research. DODIG reports that a January 2024 BUMED emergency-physician study identified concerns about skill degradation as the "number-one factor contributing to junior officer attrition" (Department of Defense Office of Inspector General, 2025, p. 15). Whether it is the survey announced in 2023 is unconfirmed. Existing findings inform questions; the pilot collects its own baseline and comparison outcomes.

Appendix F Adapted counseling materials

F1-F7 are proposed templates, not validated instruments. They adapt NHS Employers (2022, 2025), the Financial Readiness Program (n.d.) checklist, and existing Navy and DoD resources described above.

F1. Personalized compensation statement (Figure F1). Record specialty, service years, obligation end date, retirement system, household assumptions, and source dates. Compare leaving when eligible with staying to retirement over a common horizon. Separate annual after-tax cash, employer contributions, and pension present value. Include allowances, special pays, bonus obligations, insurance, and actual civilian offer terms; label benchmarks. Show sensitivity and permit corrections. This pilot statement is not represented as a Navy myPay product.

F2. Counseling checklist. Verify eligibility and benefit rules with personnel specialists; explain contingent benefits and added obligations; check civilian hours, call, productivity terms, and benefits. Review taxes and pension assumptions; ask the physician to explain the tradeoff; record corrections, unresolved questions, and follow-up. Financial counselors use a common model with specialist support.

F3. Reasons for leaving. Rank compensation, workload, deployment, clinical practice and skill maintenance, autonomy, location, and family needs; allow other reasons. Ask which change could affect the decision. Measure benefits knowledge separately from satisfaction. Historical surveys guide question design, not the participant baseline or control group.

Appendix F CDB delivery and responsibilities

F4. Delivery plan. Enhanced-group commands would be required to schedule and document an offer of review within 12 months before obligation completion and before an extension or separation commitment. An earlier routine CDB would not automatically satisfy this proposed review. Declining private financial disclosure would not affect access to career guidance.

The command Medical Corps CDB coordinator identifies eligible officers, arranges the review, and records the offer, acceptance or decline, and completion. Existing remote CDB arrangements offer a route for officers without a local coordinator (Schofer, 2024). The pilot lead maintains the audience, channel, owner, and date schedule adapted from NHS Employers (2022).

The Command Financial Specialist provides initial compensation education, the checklist, and referral. The CFS instructor guide describes education, counseling, and referral functions (Financial Readiness Program, 2021). A Fleet and Family Support Center financial counselor reviews the individualized model with specialist support; CFS designation alone is not treated as qualification to validate complex physician tax and pension estimates.

Personnel or special-pay specialists confirm eligibility, agreement dates, and added obligations. The CDB discusses clinical work, deployment, career goals, and family priorities. Detailed household figures and civilian offers remain in the private counseling process; the board receives only information the physician chooses to share.

Record offers, uptake, session duration, model version, unresolved questions, and follow-up. Train staff on the same materials and budget for their time. These responsibilities are proposed additions to existing services and require pilot authorization.

Appendix F Comparison groups and knowledge

Existing-services commands retain usual Medical Corps CDBs, financial counseling, TAP, special-pay guidance, and general compensation education. Enhanced-group commands retain all of those services and add the standardized review and documented offer. The evaluation estimates the effect of offering this package; it does not withhold existing assistance.

Record existing-service use, outside counseling, shared materials, and review uptake in both groups. Keep physicians in assigned groups even if they decline the review. Do not compare only volunteers with nonparticipants and describe that difference as a randomized treatment effect.

F5. Knowledge measure. At enrollment before the offer and six months later, administer the same five questions in both groups: identify taxable versus tax-exempt pay; identify pension eligibility; explain the bonus obligation; distinguish annual cash from pension present value; and identify an assumption that could reverse the comparison. Score zero to five using a prespecified rubric and current rules.

Test comprehension and scoring consistency before enrollment. Record missing answers and follow-up loss. Collect service stage, specialty, obligation end date, and baseline intention to stay. A knowledge increase is an intermediate outcome; it does not establish a retention effect.

Appendix F Retention evaluation and costs

F6. Outcome record. Randomize commands, balance specialty and service stage, and analyze physicians in assigned groups with command clustering. Prespecify eligibility, required sample size, exclusions, and missing-data handling. Track participation and knowledge separately from service outcomes.

The primary retention endpoint is active-service status 24 months after the original obligation ends. Record voluntary departures, involuntary losses, and reasons. Compare the offer of enhanced review with contemporaneous existing CDB and financial services; historical exit surveys cannot substitute for a randomized comparison. Document survey provenance before using any existing item or result.

F7. Cost and decision record. Count incremental CDB coordination, CFS and counselor time, physician-pay verification, model development and updates, administration, data collection, and evaluation. Record setup and recurring costs separately. Existing resources may support delivery; cost savings and cost-effectiveness must be measured.

Divide incremental cost by estimated additional physicians retained at the endpoint only if the estimated effect is positive. Report uncertainty; do not claim cost-effectiveness when retention effects are indistinguishable from zero. Prespecify the main-text five-percentage-point, confidence-interval, and replacement-cost criteria before results. These proposed policy thresholds require approval and are not validated by the source programs.

Interpret improved knowledge without retention as evidence to reassess the intervention and other departure drivers, including clinical skill maintenance and family needs. Report adverse, null, and inconclusive findings as well as favorable results.

Appendix F. Illustrative counseling handout

Figure F1 Sample compensation statement

Illustrative counseling handout—not a validated instrument

O-4 lieutenant commander | 10 service years | Age 37 | Married

Decision: leave when eligible or remain to 20 service years

Year 1 annual compensation	Stay	Leave
Gross cash pay	$276,966	$459,057
After taxes and listed premiums	$221,897	$288,445
Employer retirement contributions	$5,652	$16,560
Adjusted compensation total	**$227,549**	**$305,005**

Cash excludes employee retirement saving and other spending.

Retirement value and full-horizon results

Conditional military pension PV if staying: $517,619 after tax

Adjusted base case: leaving is ahead by $39,395 in present value.

Retiree health: +$22,878; plus Navy tax domicile: +$117,480.

With continuation pay: +$133,832; later pay penalty: +$48,096.

Positive favors staying; cumulative scenarios, not a confidence interval.

Base: O-4; CA wages; survival-weighted pension; bonus eligibility.

Discounts: cash 5%, pension 3%; pension tax 32%; no VA benefits.

5% pension discount: -$233,152. Check offer terms and family needs.

Confirm obligations, eligibility, domicile, and clinical practice needs.

Note. Rounded values from the single survival-weighted base in Appendices A-E. Pension value is separate from annual compensation; scenario increments use their stated fixed horizons. Sources: Appendix D and appendix_calculations.py, accessed October 19, 2026. Layout adapted from NHS Employers (2022, 2025); not individual advice.